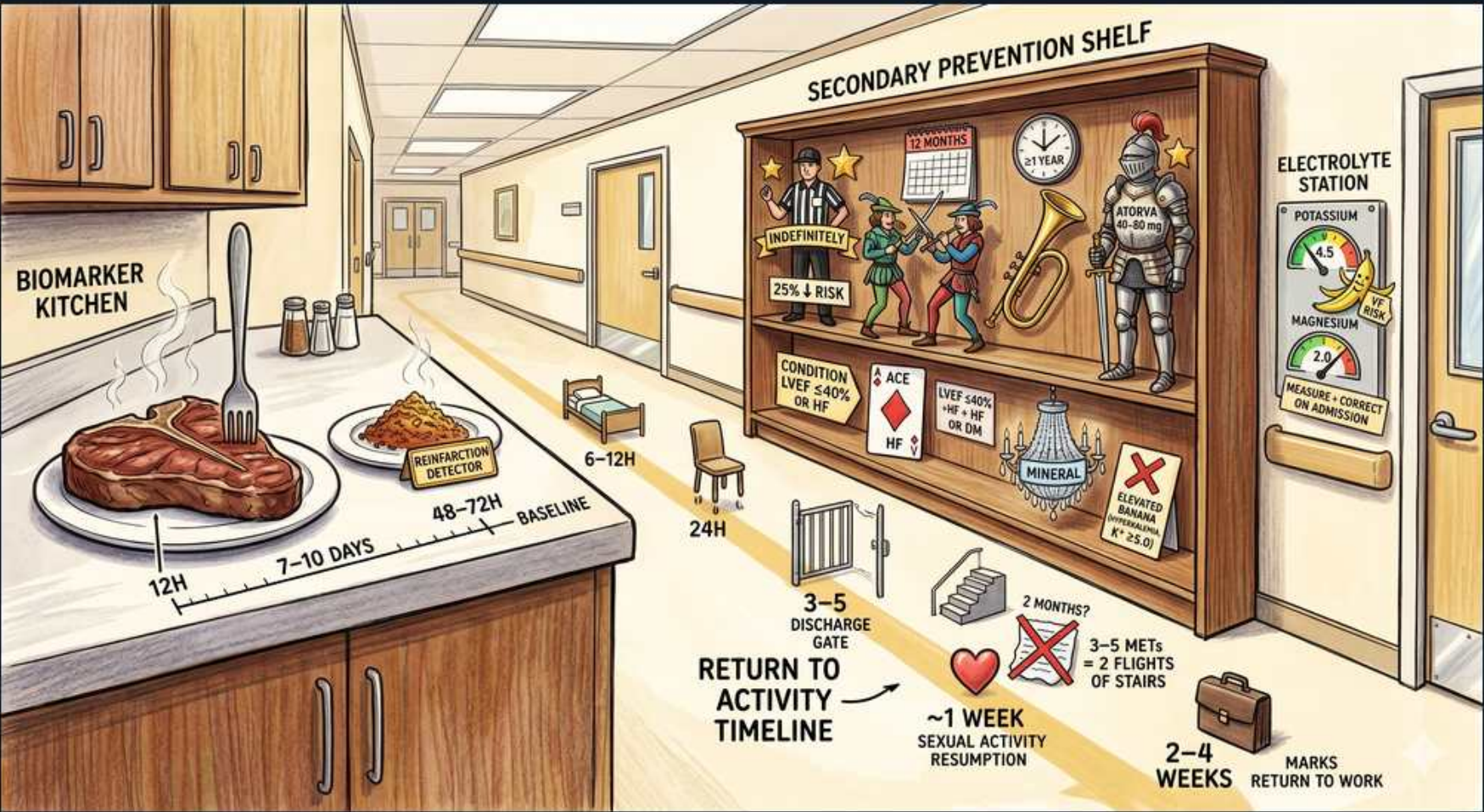


Post-MI Secondary Prevention & Return-to-Activity Timeline



1. Troponin baseline — biomarker kitchen

WHAT YOU SEE

Steak/marbling labeled BIOMARKER KITCHEN with timeline markers.

WHAT IT ENCODES

Troponin is the preferred MI biomarker: rises 3-4h after injury, peaks ~24h, and stays elevated 7-10 days, making it useful for late presentation. High-sensitivity troponin enables rapid rule-in/rule-out protocols.

BOARD TRAP

Troponin staying up 7-10 days is why it's used for late presenters; CK-MB (normalizes 48-72h) is better for detecting RE-infarction.

2. CK-MB reinfarction detector — 48-72h

WHAT YOU SEE

'REINFARCTION DETECTOR' tag, '48-72H'.

WHAT IT ENCODES

CK-MB normalizes within 48-72 hours, so a re-rise after it had fallen is the marker of re-infarction. Troponin's prolonged elevation makes it less useful for diagnosing early reinfarction.

BOARD TRAP

To detect REINFARCTION within days, use CK-MB (it has cleared) — re-rising troponin is harder to interpret because it stays elevated for ~10 days.

3. Timeline markers 6-12h / 24h / 12h

WHAT YOU SEE

Beds/markers '6-12H', '24H', '12 H' along the floor timeline.

WHAT IT ENCODES

Troponin kinetics: detectable by ~3-4h, clearly elevated by 6-12h, peaking around 24h. Serial sampling over hours confirms the rise-and-fall pattern required for the MI definition.

BOARD TRAP

A single normal troponin too early (<3-4h) does not exclude MI — serial measurement is required.

4. Secondary Prevention Shelf

WHAT YOU SEE

Cabinet 'SECONDARY PREVENTION SHELF' with referee, calendar, knight, trumpet.

WHAT IT ENCODES

Post-MI secondary prevention pillars: dual antiplatelet therapy, high-intensity statin, ACE inhibitor/ARB, and beta-blocker, plus aldosterone antagonist if EF ≤40% with HF/diabetes. All reduce mortality.

BOARD TRAP

All four pillars (antiplatelet, statin, ACEi, beta-blocker) lower mortality — leaving any out is the wrong answer; lifestyle alone is insufficient.

5. Calendar / 25% risk reduction

WHAT YOU SEE

Calendar and '25% ↓ RISK' tag.

WHAT IT ENCODES

Guideline-directed secondary prevention substantially cuts recurrent events and mortality (on the order of ~25% relative risk reduction per major class). Cardiac rehab adds further benefit.

BOARD TRAP

Don't downgrade to 'as-needed' therapy after discharge — chronic, indefinite secondary prevention is what drives the risk reduction.

6. Aldosterone antagonist — EF ≤40% + HF

WHAT YOU SEE

'CONDITIONAL: LVEF ≤40% + HF' / 'ACE ARB' panel.

WHAT IT ENCODES

Add an aldosterone antagonist (eplerenone/spironolactone) post-MI when EF ≤40% with heart failure or diabetes (EPHESUS). ACE inhibitor/ARB is indicated for all, especially with reduced EF, anterior MI, or HTN/DM.

BOARD TRAP

Aldosterone antagonists are conditional (EF ≤40% + HF or diabetes) — not given to everyone; watch potassium and renal function.

7. Electrolyte station — K & Mg

WHAT YOU SEE

'ELECTROLYTE STATION' with POTASSIUM and MAGNESIUM gauges.

WHAT IT ENCODES

Maintain potassium >4.0 mEq/L and magnesium >2.0 mg/dL post-MI to reduce ventricular arrhythmias. Aldosterone antagonists and ACEi/ARBs raise potassium — monitor to avoid hyperkalemia.

BOARD TRAP

Both extremes matter: aggressive K-raising drugs can tip into hyperkalemia — correct/monitor rather than blindly supplementing.

8. Discharge gate — within 24h

WHAT YOU SEE

Fence/gate labeled 'DISCHARGE GATE', '24H', 'ACE ... HF'.

WHAT IT ENCODES

Start oral beta-blocker and ACE inhibitor within 24h in stable patients, initiate high-intensity statin, and ensure antiplatelet therapy before discharge. Cardiac rehab referral at discharge improves outcomes.

BOARD TRAP

Therapy should begin in-hospital (within ~24h), not deferred to outpatient follow-up — early initiation is guideline-mandated.

9. Sexual activity resumption — ~1 week

WHAT YOU SEE

Broken-heart icon, '1 WEEK SEXUAL ACTIVITY RESUMPTION', '2 months?'

WHAT IT ENCODES

Sexual activity ($\approx$ 3-5 METs, equivalent to climbing 2 flights of stairs) can resume in ~1 week if the patient is stable and tolerates that exertion. PDE5 inhibitors remain contraindicated with nitrates.

BOARD TRAP

Roughly 1 week, not 'months', for stable patients — gauge readiness by exercise tolerance (2 flights of stairs), and remember the nitrate/PDE5 conflict.

10. 3-5 METs = 2 flights of stairs

WHAT YOU SEE

'3-5 METs = 2 FLIGHTS OF STAIRS' tag.

WHAT IT ENCODES

Functional capacity $\geq$ 3-5 METs (climbing two flights without symptoms) indicates readiness for sexual activity and many daily exertions. Stress testing can confirm capacity before clearance.

BOARD TRAP

METs, not calendar alone, gauge readiness — a symptom-limited 2-flight climb is the practical bedside test.

11. Return to work — 2-4 weeks

WHAT YOU SEE

'2-4 WEEKS — MARKS RETURN TO WORK'.

WHAT IT ENCODES

Most patients with uncomplicated MI return to work in ~2-4 weeks; high-exertion or high-risk occupations need stress testing and longer recovery. Driving restrictions also apply (typically ~1 week private, longer commercial).

BOARD TRAP

Return to work is weeks ($\approx$ 2-4), individualized by job demands — not months for an uncomplicated MI.

12. Crossed-out high-exertion icon

WHAT YOU SEE

Red-X 'GENTLE NO ...' activity icon.

WHAT IT ENCODES

Avoid heavy isometric exertion and very strenuous activity in the early post-MI weeks; advance gradually through structured cardiac rehabilitation. Sudden maximal exertion risks recurrent ischemia/arrhythmia.

BOARD TRAP

Early heavy lifting/strenuous exertion is discouraged — graded cardiac rehab, not abrupt return to peak activity, is correct.
